AC-09 Ventricular Fibrillation / Pulseless VT

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older in ventricular fibrillation or pulseless ventricular tachycardia.

Shockable cardiac-arrest pathway

1. Start CPR	High-quality compressions 100-120/min, depth at least 2 in, full recoil, minimal pauses; BVM with oxygen, monitor/defibrillator, and arrest-team activation.
2. Shock 1	200 J biphasic; verify manufacturer setting; immediately resume CPR for 2 minutes.
3. Shock 2	Same or higher device-appropriate energy; resume CPR immediately; epinephrine 1 mg IV/IO as soon as possible, then every 3-5 minutes.
4. Shock 3	Same or higher device-appropriate energy; resume CPR immediately.
5. Antiarrhythmic	Amiodarone 300 mg IV/IO after the third shock; may repeat 150 mg once. Alternative: lidocaine 1-1.5 mg/kg IV/IO, then 0.5-0.75 mg/kg every 5-10 min; maximum total 3 mg/kg.
6. Rhythm every 2 min	Shockable: shock and resume CPR. Organized rhythm: pulse check. ROSC: AC-10. Persistent arrest: continue cycles, Hs/Ts, and consider AC-12 when eligible.

Key treatment and handoff

Item	Approved action
Airway + capnography	Consider advanced airway and waveform capnography only when this does not interrupt compressions or delay defibrillation.
Torsades	For suspected pulseless torsades, magnesium sulfate 1-2 g IV/IO push; may repeat once. Do not use magnesium routinely for other VF/pVT arrests.

Defibrillation and compressions take priority over vascular access, medication, airway placement, movement, or a 12-lead.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Do not combine amiodarone and lidocaine without Medical Control.

- Resume CPR immediately after every shock and limit pulse checks to organized rhythms/evidence of ROSC.

- Use AC-03 for the coordinated full cardiac-arrest pathway.

Linked pathways

Pathway	Use when
AC-03	Adult cardiac arrest
AC-10	Post-resuscitation care after ROSC
AC-12	Termination of resuscitation criteria

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.